

Surgical oncology
J. Dr. Sameera Mam

Register No.
2521
Dt. & Time : 30/1/25

at 11:29 Am



GOVERNMENT TEACHING & GENERAL HOSPITAL

RAJAMAHENDRAVARAM, E.G. Dist.

CASE SHEET

UNIT

NAME OF THE PATIENT

: Alla Tata Rao 50/m
2-162

ADHAR NO

: Korukonda (MD)
Srirangapuram Eubind

FATHER / HUSBAND NAME :

Ph no- 998933 7500
Ad no- 2413 7085 0332

Government Medical College & Government General Hospital

RAJAMAHENDRAVARAM, E.G.District

Department of Pathology

Histopathology Report

B.No. : 124/25

Name of the patient : T. Suryanarayana Department : ENT

O.P./ I.P.No : 52813

Age & Sex : 77/Male

Unit/ Ward :

Date :

Clinical Details :

Specimen : Biopsy from mandibular growth.

Gross findings : Received 2 graytan bits measuring 0.5x0.5 cm and 0.2x0.2cm.

Microscopic findings : Sections studied show features of well differentiated squamous cell carcinoma.

Impression : Features of well differentiated squamous cell carcinoma.

Signature of the Pathologist

DR. P. SURESH VALLE, M.D Pathology
Regd. No. 79086
Associate Professor
Department of Pathology
GOVERNMENT MEDICAL COLLEGE
RAJAMAHENDRAVARAM.

Signature of the Pathologist

Dr. N.DIVYASREE M.D., Pathology
Regd. No. 47604
Associate Professor, Dept. of Pathology
Government Medical College
GGH, RAJAMAHENDRAVARAM.

Patient ID:	274	Patient Name:	A. TATA RAO 50Y/M
Age:	50 Years	Sex:	M
Ref. Physician:	DR. P. SPANDANA	Modality:	MR
Study Date:	13-Feb-2025	Study:	NECK

MRI NECK

TECHNIQUE:

Short & long TE, Multi ECHO in Multi PLANE

FINDINGS:

NASOPHARYNX AND OROPHARYNX

- Eustachian tube orifices and fossae of Rosenmuller appear normal on both sides.
- Longus muscles appear normal.
- No evidence of erosion of Clivus.
- Well defined heterogeneous altered signal intensity lesion noted within left half of tongue.
- Posteriorly lesion is extending upto Tonsillar fossa.
- Anteriorly upto the tip.
- Medially lesion is infiltrating genioglossus muscle.
- However no obvious extension noted beyond midline.
- Inferiorly lesion is abutting submandibular gland.
- No obvious buccal invasion noted, adjacent bone appears normal.
- Lesion is homogeneously hyperintense on T2 & STIR and isointense on STIR.
- approx measuring 2.7 x 4.5 x 5.1 cm.

LARYNX AND HYPOPHARYNX

- Hypopharynx appears normal.
- Epiglottis, pre-epiglottis space, ary-epiglottic folds, valleculae appear normal.
- False and true vocal cords appear normal. Subglottis appears normal.
- Hyoid bone, thyroid cartilage, Cricoid cartilage appear normal.
- Right and left lobes of thyroid gland appear normal.
- Internal jugular vein and carotid arteries appear normal on both sides.
- Multiple enlarged lymphnodes noted involving level Ib & II, largest with short axis diameter measuring 1.2 cm with heterogeneous signal intensity - Metastatic lymphnodes.
- Parotid glands appear normal on both sides.
- Cervical spine appears normal.

IMPRESSION: * Features S/o Ca. Tongue with metastatic cervical lymphadenopathy.

FOR CLINICAL CORRELATION & FOLLOW UP

DR. V. N. VARAPRASAD, M.D, FICR
CONSULTANT RADIOLOGIST


Dr. S. RAMYA, MD
CONSULTANT RADIOLOGIST

Department

Patient:

A. Tattaboo

Department of Pathology

Clinical Laboratory Report

MMT

Complete blood picture

Hemoglobin

Hematocrit (PCV)

RBC

MCV

MCH

MCHC

RDW-CV

ESR

Bleeding time

Clotting time

Urine sugar

Albumin

Microscopic examination (per HPF)

Epithelial cells

Pus cells

RBC

Crystals & Casts

Age & Sex

56/m

O.P./I.P. No: 7016

Date

12/2/23

81200

(Cells/ μ l)

(g/dL)
(%)
(mill/cmm)
(fL)
(pg)
(g/dL)

WBC

Differential count

Polymorphs

Lymphocytes

Eosinophils

Monocytes

Basophils

Platelets

Blood group & Rh type

Absolute Eosinophil Count (AEC)

Sickling test

59
33
08

(%)

(%)

(%)

(%)

(%)

(%)

(%)

(%)

(%)

(%)

(Cells/ μ l)

4.72 cells/cc

Body fluid analysis:

Nature of the fluid

Volume

Appearance

Total count

Differential count

Malarial Parasite

DP

GOVERNMENT MEDICAL COLLEGE/ GGH RAJAMAHENDRAVARAM

Name: A. Tadarao

Age/Sex: 50/m

OP/IP No:

Ward: PSY

Date: 18/02/2025

Investigations requested	Estimated value	Normal Range
Sugar : Fasting <u>R</u>	<u>101</u>	70 - 100 mg/dl
Post Prandial/GGT 2nd hr		<140 mg/dl
RFT: Urea	<u>33</u>	20 - 40mg/dl
Creatinine	<u>1.0</u>	0.6 - 1.2 mg/dl
Sr.Uric Acid		3.5 - 7.2 mg/dl
LFT: Total Bilirubin	<u>0.7</u>	0.3 - 1.2 mg/dl
Direct Bilirubin		0.0 - 0.2 mg/dl
SGOT		< 35 U/L
SGPT		<45 U/L
Alkaline phosphatase(ALP)		30 - 130 U/L
Total Protein		6.6 - 8.3 g/dl
Albumin		3.5 - 4.5 g/dl
Globulin		2.0 - 3.0 g/dl
Electrolytes:		
Sodium(Na)	<u>148.6</u>	136-145 mEq/L
Potassium(K ⁺)	<u>5.13</u>	3.5 - 5.1 mEq/L

Investigations requested	Estimated Value	Normal Range
Serum Amylase		22 - 100 U/L
Serum Lipase		38 - 67 IU/L
Lipid Profile:		
Total Cholesterol		150 - 200 mg/dl
Triglycerides		50 - 150 mg/dl
HDL		40 - 60 mg/dl
LDL		100 - 140 mg/dl
VLDL		5-30 mg/dl
S Calcium		8.8 - 10.6 mg/dl
S. Phosphorus		2.5 - 4.5 mg/dl
T. Magnesium		1.6-2.6 mg/dl
CSF / ASF / Pleural Fluid:		
Sugar		
Proteins		
Chlorides		
24 hours Urinary protein:		

★Suggested Clinical Correlation

Technician

Assistant professor

Prof.& HOD

Patient ID:	24	Patient Name:	A.TATA RAO 50Y/M
Age:	50 Years	Sex:	M
Accession Number:		Modality:	MR
Referring Physician:	DR.P.SPANDANA	Study:	C SPINE
Study Date:	31-Jan-2025		

M R I - CERVICAL SPINE

Sequences :

Short & long TE, Multi ECHO in Multi PLANE

FINDINGS :

Straightening of cervical spine.

Vertebral bodies and posterior elements appear normal in morphology, marrow signals.

The disc heights are normal.

Multilevel marginal osteophytes noted.

Posterior disc osteophyte complex at C3-C4, C4-C5 levels causing mild thecal sac indentation and bilateral mild neural foraminal narrowing .

Visualized CV junction shows no abnormality.

Spinal cord shows normal signal intensity.

Pre / paravertebral soft tissues appear normal.

Conclusion: * Posterior disc osteophyte complex at C3-C4, C4-C5 levels causing mild thecal sac indentation and bilateral mild neural foraminal narrowing.

FOR CLINICAL CORRELATION & FOLLOW UP

DR.V.N.VARAPRASAD,M.D, FICR
CONSULTANT RADIOLOGIST


Dr. S.RAMYA, MD
CONSULTANT RADIOLOGIST

Register No.
2521
Dt. & Time : 30/1/20

at 11.8

Aarogyasri Taken On 15/02/19
D.O. Admisison.....
D.O. Approved.....
D.O. Surgery.....
D.O. Discharge.....



GOVERNMENT TEACHING & GENERAL HOSPITAL

RAJAMAHENDRAVARAM, E.G. Dist.

CASE SHEET

AP19142545

UNIT : A Tata Rat 50/m
2-162

NAME OF THE PATIENT : KOTUKANDA
Srinagar Palay
E. G. Dist

AADHAR NO : Ph: 9989337500

FATHER / HUSBAND NAME : Ad: 2413 7085 0332

AUTHORISATION FOR ADMISSION AND TREATMENT

Name of the Patient A. Tatarao Soly I.P. No. 2521
Ward CNT Unit _____ Date 3/2/25

I Unreservedly and in my full senses give my complete consent for any diagnostic examinations biopsy transfusion or operation as may be deemed advisable in the course this hospital admission.

Cast: BC

Ph: 9989337500.

Assistant Professor
Govt. Teaching General Hospital
RAJAMAHENDRAVARAM.
Relationship of Representative

DATE :

INFORMED CONSENT

I/We A. Tatarao have been explained by the attending doctor about the nature of the patient the present condition of patient, the nature of operation to be performed, the risks involved with regard to patient's Life and the possibility of the operation anaesthesia or transfusion as may be deemed necessary.

Signature :

Relationship :

RELEASE FROM RESPONSIBILITY FOR DISCHARGE

I am leaving / taking away Smt. / Sri A. Tatarao from _____

Hospital against the advice of the attending Medical Officer, I acknowledge that I have been explained of the risk involved and hereby release attending Medical Officer and the Hospital from all responsibility for any ill effect which may result from such discharge.

Signature of Patient / Relative FAMILY

Relationship of Representative

అనుమతి పత్రము

డాక్టరు గారు నా / మా A. Tatarao

యొక్క వ్యాధి లక్షణాల గురించి ప్రస్తుత ఆరోగ్య పరిస్థితిని గురించి మరియు వారు చేయదలచిన శస్త్రచికిత్స తత్సంబంధిత ప్రాణహాని మరియు పరిణామాల గురించి విశదీకరించినారు. పై విషయము పూర్తిగా అర్థము చేసుకొని, మత్తుమందు ఇచ్చుటకు, రక్తము ఎక్కించుటకు మరియు అపరేషన్ చేయుటకు అనుమతి ఇస్తున్నాము.

సంతకము :

బాంధవ్యము :

GOVERNMENT TEACHING & GENERAL HOSPITAL

RAJAMAHENDRAVARAM, E.G. Dist.

HISTORY

NAME	AGE	SEX	Religion	Hosp.No
Service	WARD		Occupation	Income

Atata Rao 50/m

PRESENT COMPLAINT

14/2/25

Temp - 98.6°F
BP - 130/80 mmHg
SpO2 - 98%
PR - 82b/m
RR - 19b/m



ClO Pain at the
(L) Side of Neck

→ ClO generalised weakness

HISTORY OF ILLNESS

PAST HISTORY

Adv
get chest x-ray
reports
MRI Neck
report

FAMILY HISTORY

PERSONAL HISTORY

6 PMG RBS - 116 mg/dl

- Ry
- (1) C-TRAMADOL
BD
 - (2) T. Pcm 500mg
BD
 - (3) T. METRONIDAZOLE
400mg TID
 - (4) T. CIPROFLOXACIN
500mg BD
 - (5) T. PAN 400mg
 - (6) T. vit-C 100
 - (7) T. MVT 100

DS/DS

~~Adv~~

PHYSICAL EXAMINATION

15/2/28
8AM to 2PM
T - 98.6°F
PR - 82bkt
R - 12bkt
SpO₂ - 98%
BP - 110/60 mmHg

Gr pain.
Gr pain at (+) Side
of face
Gr generalized body
pain

- Rp
- ① T. Tramadol 100mg
 - ② T. Paracetamol 500mg
 - ③ T. Paracetamol 500mg
 - ④ T. Vitc 0.3
 - ⑤ T. MVI 0.3

Plan: Inj. Paclit + Carbo

15/2/25

Height - 160 cm
Weight - 52 kgs

BSA - 1.5

Inj. PACLI - 260mg

Inj. CARBOPLATIN : 600mg

6 PM GRBS - 157 mg/dl

16/2/25

9 AM.

Temp - 98.2°F
PR - 88bkt
RR - 22bkt
BP - 130/80 mmHg
SpO₂ - 99%

Temp - 98.6°F
PR - 84
RR - 24
SpO₂ - 99%
BP - 120/90

GRBS = 110 mg/dl 12:6 PM.

17/2/25

Gr Pain

1st cycle

Inj. PA CLITAXEL +

Inj. CARBOPLATIN

Chemo today

16/2/25

6 PM GRBS - 133 mg/dl

Dr. P. SPANDANA
Reg. No. 85550

Assistant Professor
Department of Radiation Oncology
GOVT. MEDICAL COLLEGE, RAJAHMUNDRY

GOVERNMENT TEACHING & GENERAL HOSPITAL
PROGRESS RECORD / DOCTOR'S ORDERS

12/2/25

(chemo today.)

du-

ALs
2. Tetmooz

T - 98.2°
PR - 84 b/min
RR - 24 b/min
SpO₂ - 97%
BP - 120/80 mmHg

GOVERNMENT TEACHING & GENERAL HOSPITAL

NURSE'S DIALY RECORD

NAME AGE SEX
SERVICE 50 M
WARD

A. Tata Rao

ALL INJECTIONS TO BE INITIALIED BY PERSON'S ADMINISTERING AND WITNESSING

Hour	Medications	Diet	Treatment Remarks
14/2/25			
6m-2pm	98.6°F 80b/m 196/m 98% 130/80mmHg GRBS - 138 mg/dl		Vitals checked T. Tramadol T. PCM T. metronidazole T. Ciprofloxacin T. Pan T. Vit C T. MVT Treatment given
8pm to 8am	98.7°F 80b/m 20b/m		
15/2/25			
8AM to 2PM	98.6°F 82b/m 196b/m 98% GRBS - 107 mg/dl	u0/60	vital Records T. Tramadol BD T. PCM BD T. Metro - QID T. CIPRO - BD T. Pan - OD T. Vit - OD T. MVT - OD

NURSES' DAILY RECORD

GOVERNMENT TEACHING & GENERAL HOSPITAL

NURSE'S DIALY RECORD

NAME AGE SEX
SERVICE WARD

A. Tata Rao

ALL INJECTIONS TO BE INITIALISED BY PERSON'S ADMINISTERING AND WITNESSING

Hour	Medications	Diet	Treatment Remarks
15/2/25 2 to 8 PM	98.6°F 20/nt 71/nt 98% 110/90 GRBS-157 mg/hl		vitals recorded
16/2/25 8 PM to 8 AM	98.6°F 20/nt 71/nt 98% 110/90	Continued	
16/2/25 9 AM	Temp PR RR SpO ₂ BP 98.6°F 88b/min 22b/min 99% 130/80 mmHg		vitals are checked & Recorded. T-Treumadol 50mg BD T-Pcm 50mg TID T-Pantop 40mg OD T-Vit-C OD T-Mv2 - OD Treatment given at 9 AM
16/2/25 2 to 8 PM	98.6°F 84/nt 21/nt 98% 110/70 mmHg		vitals checked 6 PM GRBS checked
17/2/25 8 AM to 2 PM	98.6°F 84/nt 21/nt 98% 110/70		vitals checked
17/2/25 8 AM - 2 PM	98.6°F, 84b/min, 24b/min, 99%, 120/70		vitals checked & recorded. Inj: PACUTAXEL Inj: CARBOPLATIN

NURSES' DAILY RECORD

Treatment given at 9 AM

TALVERNMENT TEACHING & GENERAL HOSPITAL

GRAPHIC SHEET

Regd. No. 2521

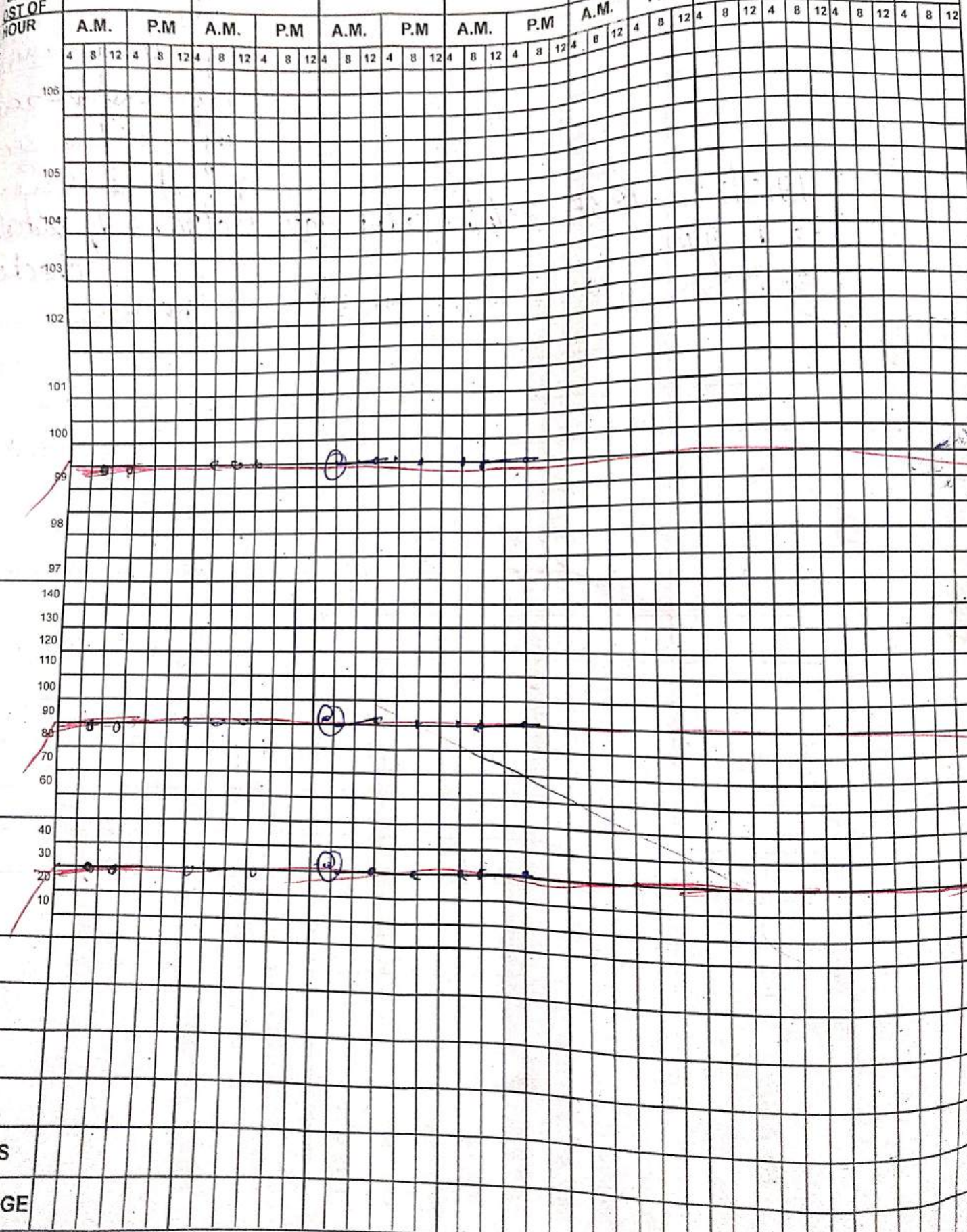
RECTAL

B.No. _____

Name: Astafar Rao Age: 30 Sex: M

Ward _____

DATE: 14/2/25 15/2/25 16/2/25 17/2/25



7A.M.
7P.M.
7A.M.
7P.M.
7A.M.
7P.M.
7A.M.
7P.M.
TOOLS
RAINAGE

DIET SHEET

Name: A. Tata Rao

Age: 80 Sex: M Ward: M

Register No.

DIET

CLASS

1. MILK
2. Sago Diet
3. Milk & Bread Diet
4. Rice Diet
5. Chapathi Diet
6. Full Diet
7. Mix Diet
8. Sippy Diet

SPECIAL DIET

Initials of Medical Officer

17/12/25	98.2°F	84lbmt	245lbmt	99%	120/80	Vitals checked
8AM to 2PM						
						Inj PA Cytaxel
						Inj CARBOPLA 40
						Chemo
						Inj - Dexa - 2amp
						my - Ondar - 200
						my - Rantae - 200
17/12/25	98.6°F	86lbmt	21lbmt	99%	120/80 mmHg	Vitals checked
2 to 8PM						
8PM to 8AM		98.6 + 86 + 2lb	120			check

Patient ID:	24	Patient Name:	A. TATA RAO 50Y/M*
Age:	50 Years	Sex:	M
Accession Number:		Modality:	MR
Referring Physician:	DR.P.SPANDANA	Study:	C SPINE
Study Date:	31-Jan-2025		

M R I - CERVICAL SPINE

Sequences :

Short & long TE, Multi ECHO in Multi PLANE

FINDINGS :

Straightening of cervical spine.

Vertebral bodies and posterior elements appear normal in morphology, marrow signals.

The disc heights are normal.

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Conclusion: * Posterior disc osteophyte complex at C3-C4, C4-C5 levels causing mild thecal sac indentation and bilateral mild neural foraminal narrowing.

FOR CLINICAL CORRELATION & FOLLOW UP

DR.V.N.VARAPRASAD,M.D, FICR
CONSULTANT RADIOLOGIST


Dr. S. RAMYA, MD
CONSULTANT RADIOLOGIST

SA-ICTC: Laboratory Report at HCTS Confirmatory Facilities (SA-ICTC)
ANDHRA PRADESH STATE AIDS CONTROL SOCIETY



Laboratory Test Report form for HCTS Confirmatory facility

SA-ICTC: PPTCT UCHH Pay

Male ☒ Female ☐ Transgender ☐

Middle name _____ First Name Taty Rao

Lab ID No.: P-17

Age: 50 (years)

Blood Drawn: 30-1-2025 (DD/MM/YY) 12:41 PM (HH:MM)

Specimen type used for testing (tick one): Serum / Plasma / Whole Blood
Date & Time of specimen tested: 30-1-25 (DD/MM/YY) 3:00 PM (HH:MM)

Column 2 and 3 to be filled only when HIV 1 & 2 antibody discriminatory test(s) used
No cell has to be left blank; indicate as NA wherever not applicable

Column 1	Column 2	Column 3	Column 4
Name of the HIV kit	Reactive/ Nonreactive (R / NR) for HIV -1 antibodies	Reactive/ Nonreactive (R / NR) for HIV -2 antibodies	Reactive/ Nonreactive (R / NR) for HIV antibodies
Test I: <u>Cadidas</u>	<u>NA</u>	<u>NA</u>	<u>Non-Reactive</u>
Test II: <u>-</u>	<u>-</u>	<u>-</u>	<u>-</u>
Test III: <u>-</u>	<u>-</u>	<u>-</u>	<u>-</u>

HIV-NON REACTIVE

- Interpretation of the result: Tick (✓) relevant
- ☒ Specimen is negative for HIV antibodies
 - ☒ Specimen is positive for HIV-1 antibodies
 - ☒ Specimen is positive for HIV antibodies (HIV-1 and HIV-2; or HIV-2 alone)
 - ☒ Specimen is indeterminate for HIV antibodies. Collect fresh sample in 2 weeks
 - ☒ Confirmation of HIV 2 sero-status at identified referral laboratory through ART centers

C. S. Suresh

Name & Signature
Laboratory technician

P.P.T.C. T. Medical Officer
Laboratory In-Charge

Government General Hospital
RAJAMAHENDRAVARAM

IT TEACHING & GENERAL LAB

Department of Microbiology GGH, Rajamahendravaram

Name : A. Katta Rao

Age : 58 Sex : M

IP/OP No. : 20250017106

Ward : General Medicine

Date : 30/11/2024

SEROLOGY REPORT

WIDAL TEST :

OT :

dilution

HT :

dilution

RA Factor :

ASO Titre :

C.R.P. :

VDRL/RPR

~~HBsAg~~ (Rapid)

~~LHCv~~ (Rapid)

HIV (Rapid)

Dengue (Rapid)

Negative

Negative



ASSISTANT PROFESSOR
DEPARTMENT OF MICROBIOLOGY
G.M.C. Rajamahendravaram
Signature

Government Medical College / Government General Hospital
 RAJAMAHENDRAVARAM, E.G. District
 Department of Pathology

Clinical Laboratory Report

patient: **A. Tata Rao**
 : **oncology.**
 Age & Sex : **58 fm.**
 Unit :
 O.P./I.P.No : **252521**
 Date : **30/11/25**

<u>blood picture:</u>	
hemoglobin (g/dL)	10.5
hematocrit (%)	5.53
erythrocytes (mill/cmm)	
reticulocytes (FL)	
erythrocyte sedimentation rate (mm/1 hr)	11
erythrocyte sedimentation rate (min)	1:25:50
erythrocyte sedimentation rate (min)	1:40:50
<u>WBC</u>	
Differential count	
Polymorphs	77
Lymphocytes	16
Eosinophils	3
Monocytes	07
Basophils	
Platelets	1,97
Blood group & Rh type	AB +ve
Absolute Eosinophil Count (AEC)	Positive
Sickling test	

Ulysis:
 sugar :
 urin :

Microscopic examination (per HPF)

casts :
 :
 :

Body fluid analysis:
 Nature of the fluid :
 Volume :
 Appearance :
 Total count :
 Differential count :
 Malarial Parasite :

Labhs/

DEPARTMENT OF BIOCHEMISTRY
GOVERNMENT MEDICAL COLLEGE/ GGH
RAJAMAHENDRAVARAM

Tata Rao

Age/Sex: *58/M.*

2521

Ward: *ONCOLOGY 20*

Date: *30-1-25*

Investigations requested	Estimated value	Normal Range	Investigations requested	Estimated Value	Normal Range
Sugar : Fasting <i>R</i>	<i>93</i>	70 - 100 mg/dl	Serum Amylase		22 - 100 U/L
Post Prandial/GGT 2nd hr		<140 mg/dl	Serum Lipase		38 - 67 IU/L
RFT: Urea	<i>21</i>	20 - 40mg/dl	Lipid Profile:		
Creatinine	<i>0.5</i>	0.6 - 1.2 mg/dl	Total Cholesterol		150 - 200 mg/dl
Sr.Uric Acid		3.5 - 7.2 mg/dl	Triglycerides		50 - 150 mg/dl
LFT: Total Bilirubin	<i>0.8</i>	0.3 - 1.2 mg/dl	HDL		40 - 60 mg/dl
Direct Bilirubin	<i>0.3</i>	0.0 - 0.2 mg/dl	LDL		100 - 140 mg/dl
SGOT	<i>63</i>	< 35 U/L	VLDL		5-30 mg/dl
SGPT	<i>32</i>	<45 U/L	S.Calcium		8.8 - 10.6 mg/dl
Alkaline phosphatase(ALP)	<i>75</i>	30 - 130 U/L	S. Phosphorus		2.5 - 4.5 mg/dl
			T. Magnesium		1.6-2.6 mg/dl
Total Protein	<i>7.5</i>	6.6 - 8.3 g/dl	CSF / ASF / Pleural Fluid:		
Albumin	<i>3.6</i>	3.5 - 4.5 g/dl	Sugar		
Globulin	<i>3.9</i>	2.0 - 3.0 g/dl	Proteins		
Electrolytes:			Chlorides		
Sodium(Na)	<i>1459</i>	136-145 mEq/L	24 hours Urinary protein:		
Potassium(K⁺)	<i>4.84</i>	3.5 - 5.1 mEq/L			

★Suggested Clinical Correlation

U. Anu
 Technician

Assistant professor
 Department of Biochemistry
 Government Medical College
 Rajamahendravaram

Prof.& HOD

Registration		 UHID: 20250017106	
Ward:/Bed: RADIATION ONCOLOGY WARD / Floor	IPD Fees : Rs. 0	NON-MLC Case	
Treating Doctor :		IP NO : 20252521	
Date Of Admission And Time:: 2025-01-30 11:29 am		Gender : Male	
Patient Name : Mr. a tatarao		Age : 50 Years	
Father's Name :			
Address :rjy,ANDHRA PRADESH,EAST GODAVARI,INDIA,		Emergency Contact Address :	
Mobile No : *****7500		Mobile No ::	
Religion : Unknown		Caste : Unknown	
Monthly Income: 0		Occupation : OTHER	
Billing Type : General		Ration Card Number :	Ration Card Type :
Admission Type : Routine		Admitting Doctor :	
Provisional Diagnosis:		Final Diagnosis :	
Prepared By : Mrs.S RAJESWARI		Signature Of Treating Consultant:	

01/2025 SAT
Name : A TATARAO
Department : Preventive Oncology
Dept No. : 2025/098/0000007
Date of Registration : 30-01-2025 10:19:28 AM
Unit : 1
Age : 50Y

Billing Type : General
Mobile No : *****500
Address : rjy, EAST GODAVARI, ANDHRA PRADESH, INDIA
Patient Type: NON MLC

Date and Time of initial assessment :
Clinical History :

Examination Findings :

Investigation :

Diagnosis :

Treatment :

Follow-up advice :

RAJAMAHENDRAVARAM
Near CTRI

OUT PATIENT RECORD
Re-visit



(atatarao_a.75@abdm)
Fee : 0.00
Sex : Male
S/O

Occupation : OTHER
Prepared by: Ms. CH SRI VALLI

of 18 Dr Spandana

Keto chronic smoker

20 yrs

of ulcer on L side body

tongue 1 month

o/e : ECOG 1

Tongue ulcer (+)

Ash
admit
ch Rad. onco

HAU-30

Answer P-17

Doctor's Name
Signature / Date

GOVERNMENT TEACHING & GENERAL HOSPITAL

NURSE'S DIALY RECORD

NAME AGE 50 SEX MA - Tata Road

SERVICE WARD

ALL INJECTIONS TO BE INITIALIZED BY PERSON'S ADMINISTERING AND WITNESSING

Hour	Medications	Diet	Treatment	Remarks
2/2/25 2-8 PM	98.6° 82 120/70 <u>GABIS - 141 mg/dl</u>	GRBS	GRBS	
2/2/25 8 PM	98.6° 82 120/70	P	Ephoin Sin	Wishes in.
2/2/25 8 PM	98.6° 82 120/70	GR	Ephoin 100y Telo Pamtop Bh and OD F. Transal 32 F. Transal 32	
3/11/25 2 PM	97.6° 80 120/70 98%			vital recorded
6 PM	<u>GABIS - 115 mg/dl</u>			treatment given
8 PM	97.6° 81 120/70 98%			GRS - 115
8 PM	97.6° 81 120/70 98%			GRS - 115
4/2/25 8 AM	98.6° SpO2 97 bpm 85 PR: 19 120/80 mmHg			Test dose 80
		Inj:	Ephoin	Vitals Recorded
		Inj:	Menocet	100mg
		Inj:	Pantap	40mg

NURSES DAILY RECORD
Githon

GOVERNMENT TEACHING & GENERAL HOSPITAL

NURSE'S DAILY RECORD

NAME	AGE	SEX	WARD	ALL INJECTIONS TO BE INITIALIZED BY PERSON'S ADMINISTERING AND WITNESSING		
A. Tata Rao 50/M						
Real to on oncology						
Hour	Medications			Diet	Treatment Remarks	
3:15	98.4°F	82 b/t	22 mt 98%		Vitals recorded	
4:30pm			110/80 mmHg		a Treatment given	
6:30pm	98.6°F	83b	22b 98%	120/90 mmHg	2 cc q.i.v	
		GIBOS 145 mg / del			6:30pm	
					Livopril given M	
					Drop	
8am-8pm	98.4°F	86 b/t	97%	120/70 mmHg	Vitals checked recorded	
3/1/25	98.6°F	87 b/t	98%	120/70 mmHg	Vitals checked Rec	
					In 3m	
2pm-3pm	98.6°F	87 b/t	98%	120/70 mmHg	Vitals Recorded.	
3/1/25	98.7°F	80/min	97%	110/70 mmHg	Vitals checked and Recorded	
8pm-8am					No Complaints Night treatments is given	
1/2/25	98.6°F	82 b/t	98%	110/70	8 GIBOS 3m	
8am					Vitals in	
2pm-3pm	98.6°F	22mt	82mt 98%	140/90	Vitals recorded	
					In 3m	
1/2/25	98.6°F	22mt	82mt 98%	120/70	8 GIBOS 3m	
8pm-8am					Vitals in	
2/2/25	98.6°F	80 b/t		100/60 mmHg	Vitals checked	

GOVERNMENT TEACHING & GENERAL HOSPITAL
PROGRESS RECORD / DOCTOR'S ORDERS

[illegible]

GOVERNMENT TEACHING & GENERAL HOSPITAL

PROGRESS RECORD / DOCTOR'S ORDERS

5/1/13

Seen by Dr. Pa. Neelam

B. P. 120/80
Clo seizure - 1 episode 9th Nov
Sensor - 9th

Ad

1) 3 ~~tablets~~ Phenytoin 100mg

2) MRG - 100mg

1/2/24
son 102cm
R.P. 110/70 cm

1/2/24

→ No clo Convulsions
→ Patient Stable

MRI Report
Brain & Neck Awaited

Rx

- (1) Inj. PHENYTOIN 100mg IV
- (2) C. TRAMADOL 50mg 10-1
- (3) T. SERRAX 100mg 10-1
- (4) T. MVT 100mg 10-1
- (5) T. PAN 40mg 10-1

2/2/24
AM
GRBS = 193 mg/dl

6pm GRBS - 141 mg/dl

GOVERNMENT TEACHING GENERAL HOSPITAL

RAJAMAHENDRAM, E.G. Dist.

HISTORY

NAME	AGE	SEX	Religion	Hosp. No
SERVICE	WARD		Occupation	Income

A. Tata Rao 50/M

Religion: Cochie
 Occupation: 1/20,000/-

PRESENT COMPLAINT :

HB Dr. Spandana

HISTORY OF ILLNESS :

ulcer in the tongue @ lat border.
 pain @ radiating to left ear.
 smoking @ (Chutta)
 Alcoholic @

PAST HISTORY :

DM- / HTN-

FAMILY HISTORY :

44

PERSONAL HISTORY :

Sluggish. Disturbed
 due to pain

Patient ID: 3659	Patient Name: A TATARAO 50Y
Age:	Sex: M
Accession Number:	Modality: CR
Referring Physician:	Study: CHEST PA
Study Date: 13-Feb-2025	

Krsnaa
DIAGNOSTICS
LTS. PG. 0000

X-RAY CHEST PA VIEW

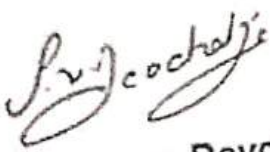
FINDINGS:

The lungs on the either side show equal translucency.
 The peripheral pulmonary vasculature is normal.
 No focal lung lesion is seen.
 Bilateral CP angles are normal.
 Both hila are normal in size, have equal density and bear normal relationship.
 The heart and trachea are central in position and no mediastinal abnormality is visible.
 The cardiac size is normal.
 The domes of the diaphragms are normal in position, and show smooth outline.

IMPRESSION:

No significant abnormality detected.
 Suggested clinical correlation.

Date: 13-Feb-2025


Dr. Prasanna Devchake
 M.B.B.S., D.M.R.E.
 Reg.No. 2005/03/1611

In Association With
Krsnaa Diagnostics Ltd.
 OPERATIONAL FROM KASHMIR TO KANYAKUMARI

Reporting Sta